Asthma Management – Workplace Scenarios (Residential Aged Care)

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Five branching scenarios, one per lesson, built around conversations between a nurse and the head nurse. Each one ends with what to do on the floor.

How the scenarios work

The learner plays Mei, a registered nurse three weeks into a new role. Maggie, the head nurse, coaches her through five situations with two residents who have asthma.

Character	Role
Mei Chen	Registered nurse, new to the home (learner's role)
Maggie O'Brien	Head nurse (clinical care coordinator), 20 years in aged care
Harold Peters	Resident, 84, has had asthma since childhood
Joan Fitzgerald	Resident, 79, moved in last year with an asthma action plan from her previous GP

The setting is Wattle Wing, a residential aged care home in Melbourne's eastern suburbs. The scenarios run in order across one month, so Harold and Joan's stories build from lesson to lesson.

Each lesson follows the same pattern:

1. **Situation:** a short conversation between Mei and Maggie that ends on a dilemma.
2. **Decision point:** three options for what Mei does next.
3. **Consequences:** what happens after each option.
4. **Debrief:** the stronger choice and why.
5. **On the floor:** three to five actions the learner can use on their next shift.

Clinical review note: all clinical content is aligned with National Asthma Council Australia guidance at the time of writing. Before release, have a registered nurse or respiratory educator check it against the current Australian Asthma Handbook and your

organisation's policies. Nurses always follow the resident's medication chart, written asthma action plan and their own scope of practice.

Lesson 1 – What asthma is

Learning outcome: recognise the signs of asthma in an older resident and know that night-time symptoms mean asthma is not well controlled.

Situation: "It's probably just his age"

[Nurses' station, 6:45 am handover. Maggie reads the overnight notes. Mei has just arrived.]

MAGGIE: Morning, Mei. Anything from overnight I should know about?

MEI: Mr Peters was coughing again around 2 am. That's the third night in a row. The night nurse wrote that it's probably just his age.

MAGGIE: Was he wheezy? Any chest tightness?

MEI: A bit of a wheeze. He told her his chest felt "like a belt". He's got COPD too, hasn't he?

MAGGIE: Check his chart. Harold's diagnosis is asthma, since he was a boy. No COPD.

MEI: Oh. I always think of asthma as a kids' thing.

MAGGIE: Plenty of our residents have it, and it's easy to miss in older people. We blame the breathlessness on age, or the heart. What's actually happening is the airways get inflamed and swollen, the muscles around them tighten, and they make extra mucus. Less room for air.

MEI: So the cough is his airways reacting.

MAGGIE: Exactly. Wheeze, cough, chest tightness, shortness of breath. Any of those at night or first thing in the morning is telling us something.

Decision point: What should Mei do with the overnight report?

- **A:** Record a settled night with a minor cough. A bit of coughing is normal at his age.
- **B:** Document the three-night pattern, check Harold's asthma action plan and ask for a GP review.
- **C:** Ask the night staff to give his reliever at 2 am every night so he can sleep.

Consequences

A: [Friday, 3 am. Ambulance lights outside Harold's window.] **MAGGIE:** Three nights of warning signs were sitting in our notes. Harold's in hospital with a severe flare-up.

B: [Later that day.] **MAGGIE:** Good catch. I've put him on the GP's list for this afternoon. [One week later.] **MEI:** The GP stepped up his preventer. No night cough for five nights now.

C: MAGGIE: That treats the symptom and hides the pattern. If Harold needs his reliever every night, his asthma isn't controlled. That's a reason for a review, not a routine.

Debrief

Stronger choice: B. Asthma is a long-term condition where the airways become inflamed and narrow. Night-time or early-morning symptoms are a sign it is not well controlled, so they need to be reported, not normalised.

- **A** is common because breathlessness in older people is often blamed on age, heart failure or COPD.
- **C** is well-meant, but regular reliever use without review masks worsening asthma.
- **B** recognises the signs, checks the resident's actual diagnosis and plan, and escalates for review.

On the floor

- Know which residents in your area have asthma. Check the diagnosis, not just the symptoms.
- Watch for the four main signs: wheeze, cough, chest tightness and shortness of breath.
- Treat symptoms at night or early morning as a warning sign and document the pattern.
- Report repeated symptoms or increased reliever use to the RN in charge and request a GP review.

Lesson 2 – Triggers

Learning outcome: identify common asthma triggers in an aged care home, reduce exposure, and record each resident's triggers in their care plan.

Situation: "She was fine at breakfast"

[Corridor outside Joan's room, 10:15 am. A housekeeping trolley is parked outside. The window in Joan's room is wide open. Joan is coughing in her armchair.]

MEI: Maggie, Mrs Fitzgerald is coughing and a bit wheezy. She was fine at breakfast.

MAGGIE: What's changed since breakfast?

MEI: Housekeeping has just done her room. It smells really strongly of lavender spray.

MAGGIE: There's one. Strong scents and aerosol sprays are common triggers. Anything else going on today?

MEI: Housekeeping opened the window to air the smell out.

MAGGIE: Have you seen the air quality notice on the staff board? There's smoke haze forecast from planned burns this afternoon.

MEI: So opening the window lets another trigger in.

MAGGIE: Triggers stack up. Colds and flu, smoke, cold air, dust, strong smells, some medicines, even reflux or stress. Every resident's list is different, and it should be in their care plan.

MEI: Joan's care plan just says "asthma". No triggers listed.

Decision point: What should Mei do?

- **A:** Leave the window open to clear the smell, and give Joan her reliever if she gets worse.
- **B:** Close the window, follow Joan's action plan for her symptoms, ask housekeeping to avoid sprays, and add her triggers to the care plan.
- **C:** Ask housekeeping not to use sprays in Joan's room, and leave it at that.

Consequences

A: [3 pm. Haze drifts through the open window.] **MEI:** She's needed her reliever twice since lunch. **MAGGIE:** We added a second trigger on top of the first.

B: [Joan settles after her reliever.] **MAGGIE:** Nicely done. [Next week, at handover.] **MEI:** Joan's care plan now lists aerosols and smoke. Housekeeping has switched to unscented products in Wattle Wing, and windows stay shut on smoke-alert days.

C: [Saturday. Joan wheezes in the lounge after it's sprayed.] **MAGGIE:** Her room was fine, but nothing was written down. The weekend team had no idea.

Debrief

Stronger choice: B. Reducing exposure to triggers is one of the simplest ways to prevent asthma flare-ups. Recording them makes the whole team part of the solution.

- **A** treats symptoms but ignores the cause, and makes things worse on a smoke day.
- **C** helps in one room, but without documentation the knowledge leaves with the shift.
- **B** manages symptoms now, removes the triggers, and records them so every shift knows.

Common triggers in residential aged care include respiratory infections, smoke from bushfires or wood heaters, cold air, dust and mould, strong scents and cleaning products, and some medicines. In Melbourne, spring also brings the risk of thunderstorm asthma on high-pollen days.

On the floor

- Ask "what's changed?" when a resident's breathing changes suddenly.
- Check air quality and pollen alerts on high-risk days, and keep windows closed when smoke or pollen is high.
- Use unscented products around residents with asthma and avoid aerosol sprays.
- Record each resident's known triggers in their care plan and mention them at handover.
- Encourage flu and other recommended vaccinations, since infections are a major trigger.

Lesson 3 – Medications and devices

Learning outcome: tell relievers and preventers apart, check that a resident can use their inhaler properly, and care for spacers and the mouth after a preventer.

Situation: "The medicine's just sitting in his mouth"

[Medication room, 8:40 am, after the morning round. Mei is restocking the trolley. Maggie leans in the doorway.]

MAGGIE: How's Harold managing the new preventer the GP started?

MEI: Not well. It's a dry powder inhaler. He breathes out into it first, and when he breathes in it's quite weak.

MAGGIE: Dry powder needs a strong, fast breath in. If he can't manage that, most of the medicine stays in his mouth and throat.

MEI: That might explain something else. He said his mouth's been sore. There are white patches on his tongue.

MAGGIE: Did anyone get him to rinse and spit after his preventer?

MEI: I don't think so.

MAGGIE: Preventers contain a steroid. Without a rinse, gargle and spit afterwards, residents can get oral thrush. We'll ask the GP to look at his mouth.

MEI: And his blue reliever goes through his spacer, right?

MAGGIE: Yes, if he has one. A puffer with a spacer gets more medicine into the lungs for most older people. Who cleaned the spacer?

MEI: Night staff wiped it out with a towel.

MAGGIE: Warm soapy water, no rinse, then let it air dry. Towel drying causes static, and the medicine sticks to the sides.

Decision point: What should Mei do about Harold's preventer?

- **A:** Keep using the dry powder inhaler and coach Harold to breathe in harder.
- **B:** Document the technique problem, ask the GP and pharmacist about a device that suits him, and add rinse-and-spit to every preventer dose.
- **C:** Skip the preventer when he struggles with it, since he still has his reliever.

Consequences

A: [Two weeks later, handover.] **MEI:** His night cough is back. The dose counter shows every dose was given. **MAGGIE:** Given, but not getting to his lungs.

B: [After the pharmacist's review.] **MEI:** Harold's on a preventer puffer with his spacer now, and the chart says "rinse and spit". **MAGGIE:** And his mouth is healing. That's a good week's work.

C: MAGGIE: Without his preventer the inflammation comes back. The reliever opens the airways for a few hours, but it doesn't treat the cause.

Debrief

Stronger choice: B. A medicine only works if it reaches the lungs. Checking technique and choosing the right device are as important as the medicine itself.

- **A** keeps the medicine on the chart, but it isn't reaching the lungs.
- **C** stops the treatment that prevents attacks. Never skip a prescribed dose without speaking to the prescriber.
- **B** fixes the real problem, and protects Harold's mouth.

Medicine type	What it does	Key points
Reliever (blue/grey puffer, e.g. salbutamol)	Quickly relaxes the airway muscles to ease symptoms	Works within minutes; needing it more often is a warning sign
Preventer (inhaled corticosteroid)	Reduces inflammation over time to prevent attacks	Take every day, even when well; rinse, gargle and spit afterwards
Combination inhaler (preventer plus long-acting reliever)	Controls inflammation and opens airways	Some are used as both preventer and reliever; follow the action plan

On the floor

- Watch each resident use their inhaler at least once and record any technique problems.
- Use a spacer with puffers wherever the resident has one.
- Make "rinse, gargle and spit" part of every preventer dose.
- Wash spacers in warm soapy water, don't rinse, and let them air dry.
- Check dose counters and expiry dates, and report residents who can't manage their device to the GP or pharmacist.

Lesson 4 – Action plans

Learning outcome: read a written asthma action plan, recognise when a resident is moving into the "getting worse" stage, and know when a plan needs to be reviewed.

Situation: "It's signed by a doctor"

[Head nurse's office, Monday 2 pm. Monthly care plan reviews. Joan's file is open on the desk.]

MAGGIE: Mei, can you pull out Mrs Fitzgerald's asthma action plan for her review?

MEI: Here it is. It's dated 2022, from her old GP before she moved in.

MAGGIE: Four years old. Talk me through it.

MEI: Daily preventer twice a day, reliever when needed. Then a section for when asthma is getting worse, and one for emergencies.

MAGGIE: How often has she needed her reliever this week?

MEI: Four times. Twice on the smoke day, and twice since.

MAGGIE: So which section of the plan is she in?

MEI: The "getting worse" section. It says to increase her preventer and see her doctor. But the dose it gives is for a preventer she isn't on anymore.

MAGGIE: That's the problem with an old plan. Its instructions don't match her medication chart.

Decision point: What should Mei do?

- **A:** Follow the plan as written. It's signed by a doctor.
- **B:** Manage today's symptoms using her current medication chart, ask the GP to review her today, and request an updated written action plan.
- **C:** Increase her current preventer herself, to match what the old plan says.

Consequences

A: [Wednesday, 7 am.] **MAGGIE:** Night staff gave a dose from the old plan that doesn't match her chart. We're writing a medication incident report this morning.

B: [Thursday.] **MEI:** The GP reviewed Joan yesterday and wrote her a new action plan. It's in her file, a copy is in the medication folder, and I flagged it at handover. **MAGGIE:** Now every shift is working from the same instructions.

C: MAGGIE: Changing a dose isn't our decision. The action plan is the GP's instructions, and these ones are out of date. Ring the GP first.

Debrief

Stronger choice: B. A written asthma action plan only helps if it is current and matches the resident's medicines. Rising reliever use is the signal to act early.

- **A** feels safe because it's signed, but an out-of-date plan can lead to medication errors.
- **C** goes outside scope. Dose changes must come from the prescriber.
- **B** keeps the resident safe today and fixes the plan for every shift.

A written asthma action plan is prepared by the resident's doctor. It usually covers daily medicines, signs that asthma is getting worse and what to do, and what to do in an emergency.

On the floor

- Check that every resident with asthma has a written action plan, and know where it is kept.
- Compare the plan with the current medication chart. If they don't match, tell the RN in charge and contact the GP.
- Treat needing a reliever more often than usual as a sign asthma is getting worse.
- Ask for a plan review at least yearly, after a flare-up or hospital visit, and whenever medicines change.
- Include any action plan changes in handover.

Lesson 5 – Asthma attack first aid

Learning outcome: recognise a severe asthma attack, call an ambulance at the right time, and give asthma first aid with a reliever and spacer.

Situation: "Can't... get... my breath"

[Dining room, 12:15 pm. Lunch service. Harold is hunched forward at his table, gripping the edge. His breathing is fast and noisy. Mei hurries over. Maggie is at the servery.]

MEI: Maggie! Harold's struggling.

HAROLD: Can't... get... my breath.

MEI: Harold, can you tell me what's happening?

HAROLD: [shakes his head, gasping] Chest... tight.

MAGGIE: [arriving] He can only get out a word or two. Harold, we're right here with you. Mei, what's your first move?

Decision point: What should Mei do first?

- **A:** Help Harold to his room so he can lie down, then call the GP.
- **B:** Keep Harold sitting upright and stay with him. Call 000 now, because he can't speak comfortably, and start asthma first aid with his reliever and spacer.
- **C:** Grab his reliever and give him several puffs quickly, one straight after another, to get it into him fast.

Consequences

A: [Corridor. Harold stops, leaning on the rail, unable to walk further.] **MAGGIE:** Stop. Walking and lying flat make breathing harder. Sit him down here. We start first aid now and call an ambulance.

B: [Maggie dials 000 while Mei shakes the reliever and fits the spacer.] **MAGGIE:** Ambulance is on its way. [Mei gives one puff at a time, counting four breaths after each.] **MEI:** That's four puffs. Waiting four minutes. [Four minutes later.] **MEI:** Still struggling. Four more puffs. [Paramedics arrive.] **MAGGIE:** Eight puffs of salbutamol through a spacer, the first at 12:17. Here's his action plan and medication chart.

C: [Mei presses the puffer three times in a row.] **MAGGIE:** Slow down. Fired off that fast, most of it hits the back of his throat. One puff, then four breaths, every time.

Debrief

Stronger choice: B. Harold could only say a word or two, which is a sign of a severe attack. That means calling an ambulance straight away, as well as starting first aid. Staying calm and keeping him upright helps him breathe.

- **A** delays treatment, and moving or lying down makes breathing harder.
- **C** has the right medicine but the wrong technique, so less reaches the lungs.
- **B** gets help on the way and starts effective first aid immediately.

Asthma first aid for adults using a blue/grey reliever puffer, from the National Asthma Council Australia chart:

1. Sit the person upright. Stay calm and reassure them.

2. Give 4 separate puffs through a spacer: 1 puff, then 4 breaths, repeated until 4 puffs are given.
3. Wait 4 minutes, staying with them.
4. If they still can't breathe normally, give 4 more puffs. If breathing is still not normal, call 000 and keep giving 4 puffs every 4 minutes until the ambulance arrives.

Call 000 straight away if the breathing problem is severe, the person is drowsy or looks blue around the lips, is getting worse or not improving, or you're not sure what to do. If someone with asthma and a severe allergy has sudden breathing trouble, give adrenaline first, then the reliever. If a resident's action plan uses a combination inhaler for first aid, follow their plan.

On the floor

- Know where each resident's reliever and spacer are kept, and where the first aid kit reliever is.
- Sit the resident upright and stay with them. Don't walk them anywhere.
- Call 000 early. You can always cancel, but you can't get those minutes back.
- Give one puff at a time with four breaths each, and note the time of every dose.
- Afterwards, document what happened, inform the GP and family, and ask for an action plan review.

Sources

- First aid for asthma ages 12+ chart, National Asthma Council Australia
- First aid for asthma ages 12+ chart (PDF), National Asthma Council Australia
- Australian Asthma Handbook, National Asthma Council Australia, for clinical review before release